



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : حنان انور محمود محمد
المحافظة : القليوبية الحي / المدينة : شبرا الخيمة ثان العنوان : 0
رقم البطاقة : 27001181300068 رقم التليفون : 01222796040 تاريخ تسجيل الإستمارة : 21-09-2026
العمر (تاريخ الميلاد) : 18-01-1970 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي
التوقيع: (-----) المقر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : جراحات أورام تدخل جراحى لاستئصال تحفظى وتكميلي ورفع للثدى قسم العلاج :
+استئصال الغدد الليمفاوية و/او الغدد الحارسة [13156]

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : ----- التوقيع : -----
الاسم : د/ندى حسن الوظيفة : ----- التوقيع : -----
الاسم : د/نرمين رمضان الوظيفة : ----- التوقيع : -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. ----- /د.أ. ----- /د. ----- /د. -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : ----- قسم : -----
جهة العلاج : ----- نفقات : -----
ملاحظات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: حنان انور محمود محمد National ID: 27001181300068	
---	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

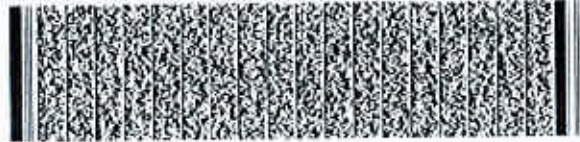
نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد أنه
--

Breast-conserving and completion surgery with mastopexy plus axillary lymph node dissection and/or sentinel lymph node biopsy
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



۱۵-۳۳
۱۵-۳۳





DMS

- ماله ٢١



MDT FORM

18/12/2025 G

Name	منال انور محمود	MRN	CVV2/CO
Age	55 years	ID	
Menopausal status	past Menopausal	Phone	012 2279604
NRS SCORE		Marital status	Divorced
BMI		Family history	+ve
CO morbidity	M: Free S: tonsillectomy & Appendectomy	REASON OF MDT	New Case

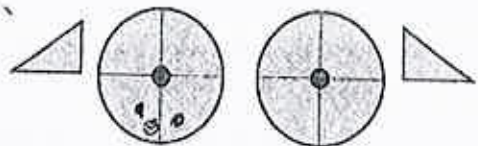
Clinical Data

BX: IDC G1e DCIS

ER: 8/8 PR: 3/8 HER2: +2 → not Ki67: 25%

Sonamammography: Anteroposterior
SM: three irregular masses
① largest at 7 o'clock.

Mets Workup: 10x7mm
② lesion seen at 8 o'clock
7x6mm 8mm deep from skin.
two suspicious axillary LN level 1



TNM
3 1 0

Recommendations:

CEM: Multiple Enhancing lesion seen at 5-8 o'clock
the largest at 7 o'clock: 3x2.3cm
the oocyte area: 6.6x5.1x8.8cm
its concentricity: Right Pathological LNod.

MDT OUTCOME & PLAN

CTCAP: Free

From NAE after breaking
then CBs
4 AC → 12 taxol weekly

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
Dr. H. H. H.	Dr. H. H. H.				

Patient Name : الأستاذ / حنان النور محمود محمد
Patient No. : 27001181300068
Path No. : 2828/25
Sex : Female
Age : 55 Y
Referring Doc :

Request No. : 13009057
Request Date : 14/09/2025
Verification Date : 25/09/2025
Financial Category : جوماتاف رومي - مبادره
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

Diagnosis

Right breast mass, Tru cut needle biopsy (TCNB):
Invasive duct carcinoma, Grade 2, WITH WIDE MICROPAPILLARY FEATURES & FOCAL
MUCINOUS FEATURES.

IPT:

- * ER: Positive (score 8/8).
- * PR: Positive (score 5/8).
- * HER2: Equivocal (score 2+).
- * SISH (Not amplified)
- * Ki67: 25 %

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

ISH groups:

The ratio is calculated and the results is interpreted as:

Verified by :

Dr.Heba Hasan

Printing Date : 29/09/2025 09:48:13

Page 2 of 3

Printing By : Mrs.Mayada Hegazy



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأستاذ / حنان انور محمود محمد
Patient No. : 27001181300068
Path No. : 2828/25
Sex : Female
Age : 55 Y
Referring Doc :

Request No. : 13009057
Request Date : 14/09/2025
Verification Date : 25/09/2025
Financial Category : جوسفات روسي - مبادره
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

- Group I (Positive): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is more than 4.
- Group II (Negative): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is less than 4. (Recounting give the same result).
- Group III (Positive): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 6 (Recounting give the same result).
- Group IV (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 4 but less than 6. (Recounting give the same result). Presidential Initiative for Women's Health- Pathology Lab Templates
- Group V (Negative): HER2/CEP17 ratio is less than 2

Signature
Dr.Walaa Ibrahim Ghoneim
Dr.Heba El-shafeey

Verified by :
Dr.Heba Hasan

Printing Date : 29/09/2025 09:48:13

Page 3 of 3

Printing By : Mrs.Mayada Hegazy



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mistr El Qadema - Cairo, Egypt

Patient ID: 90429	Patient Name English: Hanan Anwar Mahmoud
Age: ١٠ سنة ١٠ شهر	2025/12/03 11:13:34 AM :Report Date

Contrast enhanced Spectral Mammography

CLINICAL SUMMARY: A 55-year-old female patient with right breast pathologically proven IDC grade II (+ve ER, +ve PR, -ve HER2 and Ki67= 25 %)

Technique

CC & MLO low energy & recombined (post contrast) images

FINDINGS:

Background Uptake: mild nodular background enhancement

ACR: C (scattered fibro-glandular densities)

RIGHT BREAST:

- Low energy (Non contrast) images: Multiple LOQ irregular spiculated dense mass lesions.
- Recombined (Post contrast) images show the following:
 - ❖ Corresponding multiple irregular shaped enhancing lesions of marked conspicuity (seen between 5-8 o'clock by complementary US).
 - ❖ The largest lesion seen at 7 o'clock outer zone measuring 3 x 2.3 cm.
 - ❖ The nearest lesion to the nipple is seen at about 4 cm from it.
 - ❖ They occupy an area measuring 6.6x 5.1x 8.8 cm at its AP, TS and CC diameters.
- Normal skin / nipple / areolar complex.
- Complementary US: A right pathological axillary node with focal cortical thickness(6mm).

LEFT BREAST:

- Low energy (Non contrast) images No masses, asymmetry or distortion or suspicious microcalcifications.
- Recombined (Post contrast) images show no pathological uptake.
- Normal skin / nipple / areolar complex.
- Complementary ultrasound: No solid or cystic lesions.
- Nonspecific axillary nodes with preserved fatty hilum.

IMPRESSION:

RIGHT BREAST: Right lower breast multiple enhancing lesions extending between 5 and 8 o'clock with ipsilateral pathological axillary lymph node as described above (BIRADS 6= pathologically proven IDC grade II)

LEFT BREAST: Normal study (BIRADS 1= normal)

Best regards,

Dr. Heba Ahmad . MD

Dr. Sherhan Fakhry .MD

mult. focal for biopsying
+ MACT, then CBS.

Patient Name: Hanan Anwar Mahmoud Mohamed	Date : 06/09/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 55 Year(s)
Medical ID: 7930	Pat. ID: 111306

BIATERAL MAMMOGRAM AND BREAST ULTRASONOGRAPHY

Indication: Palpable right breast mass.

- Breast density: heterogeneously dense... Matching ACR C
- Spiculated opaque masses are seen on the right breast at lower region overlapping on breast parenchyma.
- No detected suspicious mass density, architectural distortion or pathological micro-calcification in the left breast.
- Normal skin and nipple on both sides.

By complementary breast ultrasound:

Right Breast:

- Homogenous (predominantly glandular) echo-texture.
- Three Irregular hypoechoic masses are seen in the right breast as follows:
 - Largest lesion is seen at 7 o'clock position, zone 2, it measures 10x7 mm being 8 mm deep to the skin.
 - A lesion seen at 6-7 O'clock position zone 1 measuring 9x8 mm, being 18 mm from the largest lesion and 7 mm deep to the skin.
 - A third lesion seen at 6 O'clock position zone 2 measuring 7x6 mm being 4 cm from the first lesion and about 2 cm from the second lesion and 8 mm deep to the skin.
- Normal right skin and nipple and chest wall muscles.
- No detected suspicious solid or cystic masses in the left breast.
- Two suspicious looking level I right axillary lymph nodes, one is rounded with lost fatty hilum measuring 6 mm and the other shows focal cortical thickening measuring 6 mm.

Left Breast:

- Homogenous (predominantly glandular) echo-texture.
- No detected suspicious solid or cystic lesions.
- No detected significant left axillary LNs.

CONCLUSION:

- Right breast: three suspicious looking masses with pathological axillary lymph nodes suggestive of multicentric process, for histopathological correlation and further assessment by contrast-enhanced study BIRADS 5 Category.
- Left breast: Normal study..... BIRADS 1 Category.

Yara Mohamed, MD

Patient Name: Hanan Anwar Mahmoud Mohamed

Exam : CT CAP+C (CT Chest Abd. Pelvis)

Medical ID: 2774/2025

Date : 13/11/2025

Age : 56 Year(s)

Pat. ID: 111306

MULTISLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS WITH CONTRAST

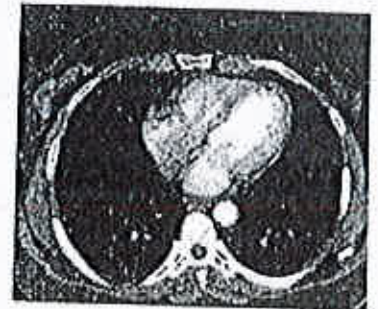
TECHNIQUE: Multi slice axial CT images of the chest, abdomen and pelvis were obtained followed by multiplanar reconstruction of the source images. I.V contrast administration.

FINDINGS:

****Given history of right breast cancer:**

CECT CHEST:

- Right breast shows dense parenchyma with multiple soft tissue lesions, largest one measures about 2.5 x 1.5 cm.
- Multiple right axillary level I & II lymph nodes, largest one measures about 12 x 9 mm.
- Bilateral basal atelectasis.
- No cavitory lesions or bronchiectatic changes.
- No CT evidence of hilar or mediastinal lymph node enlargement.
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.



CECT ABDOMEN AND PELVIS:

- The liver is average in size showing homogenous CT density with no enhancing focal lesions or biliary radicles dilatation.
- The gall bladder shows average wall thickness. Its cavity shows no dense stones.
- Normal CT appearance of the spleen, pancreas and both kidneys.
- Regular filling of the urinary bladder showing no stones, masses or diverticulae.
- No abdominal or pelvic pathologically enlarged lymph nodes.
- No free collection.
- Calcified uterine lesion measures about 2.5 x 1.5 cm...Likely Fibroid.

THE SCANNED SKELETON:

- The scanned skeleton shows no definite worrisome lytic or sclerotic lesions.
- No vertebral structural collapse or extra-osseous soft tissue component.

Opinion:

- Multiple right breast soft tissue lesions.
- Multiple right axillary level I & II lymph nodes.
- No hepatic / pulmonary or osseous lesions could be detected.

Dr. Heba El Balsky, MD

Lecturer and Consultant of Diagnostic and Interventional Radiology,
NLI-Menoufia University

جوستاف
روستاف
9 صفر 1448



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

المنشآت الصحية
MDT FORM

09/15/2026

MDT FORM

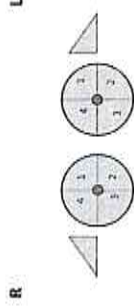
Name:	Hanan Anwr Mahmoud Mohamed	MRN:	1053
Age:	57 Y	ID:	27001181300068
Menopausal Sta		Phone:*	01222796040
NRS SCORE:		Marital Status:	
BMI:		Family History:	
Gender		Reason Of MDT:	
CO Morbidity:		post NACT	
Surgical History:** miscarriages, appendectomy + tonsillectomy		ECOG Performance Status	0 1 2 3 4

Clinical Data:

BX: PR: 5/8 HER2: not amp KI67:

History:

POSTMENOP RT BREAST CANCER CT2 N1 MO CLIPS INSERTED EF 62
ER8/8 PR5/8 HER2 SISH NOT AMPLIFIED
pt received 4 AC, 4 TAXANE, last on 24/8 with good clinical response
BSM requested, for surgical assessment



Sonomammography:

Biomarker/Molecular

Mets Workup:

Clinical Staging:
ct2 n1 mo

Recommendations:

FOR RT WLE +SLNB (WIRE LOCALIZATION OF 3 CLIPS)

MDT Outcome & Plan

FOR RT WLE +SLNB (WIRE LOCALIZATION OF 3 CLIPS)

Right Side Left Side
Q1 Q2 Q3 Q4 Q1 Q2 Q3 Q4

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
نخاع عبد العزيز	محمد نوراوي		



Patient's Name: Hanan Anwar Mahmoud Mohammed
Date: 2026-09-12
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

**FOLLOW UP BILATERAL DIGITAL SOFT TISSUE MAMMOGRAPHY WITH
HIGH RESOLUTION BREAST SONOGRAPHY REVEALED**

CLINICAL SUMMARY:

A known case of right carcinoma , under NACT and hormonal therapy, for follow up

FINDINGS:

LEFT BREAST:

- Dense breast (ACRc).
- No mass, spiculation or suspicious micro calcifications
- tissue marker is seen
- Complimentary **U/S partial response of the previously seen lesions** demonstrated the underlying hypoechoic mass located at 7 o'clock measuring about 9x8 mm , the nearest lesion to the nipple seen at 5 o'clock measuring about 7x 4 mm ,another 8 o'clock para areolar lesion is seen measuring about 1.2x7 mm
- No definite pathological axillary nodes are seen in today's study.

RIGHT BREAST:

- heterogenously dense, (ACR B).
- No mass, spiculation or suspicious micro calcifications, this was confirmed by an U/S examination.
- Normal skin/nipple/areola complex.
- Non specific axillary nodes are noted.

CONCLUSION:

right breast (BIRADS 6): rather stationary course of the residual malignant mass as described

left breast (BIRADS 1): normal study

Best regards,
Dr. Dina sayed, MSC ,CUDBIT